| DPP | DEPARTMENT POLICY AND PROCEDURE | | |
|---|---|---|---|
| Title | **ASSESSMENT AND REASSESSMENT IN PICU** | | |
| Department | **Pediatric Intensive Care Unit** | | |
| Applies to | **All PICU Medical and Nursing Staff** | | |
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## 1.0 PURPOSE

1.1. To conduct a comprehensive initial nursing assessment and periodic reassessment of the patient.

1.2. To utilize this assessment data as the basis for identifying the patient's needs and for planning appropriate nursing care.

1.3. To document pertinent findings and to provide other member of the healthcare team with information that will improve patient care.

## 2.0 DEFINITION/ABBREVIATION

2.1. **Nursing Assessment**- is the systematic collection of subjective and objective data to determine patient's current health status and identify actual and potential health problems. It is to be performed periodically to analyse the health condition of patient by a registered nurse.

2.2. **PICU**: Pediatric Intensive Care Unit

## 3.0 RESPONSIBILITY

3.1 **ICU Specialist:** responsible for comprehensive assessment and reassessment of the patient.

3.2 **Head Nurse** responsible in ensuring that all staff is doing assessment and reassessment.

3.3 **Staff nurses** must demonstrate proficiency in performing the assessment and reassessment with accordance to the policy and guidelines as mentioned.

## 4.0 POLICY

4.1. A comprehensive nursing assessment shall be carried out by conducting a complete assessment of the patient through interview, observation, inspection, auscultation, palpation, percussion and review of reports and records.

4.2. Nursing assessment and reassessment has to be performed by the registered nurse.

4.3. The nursing assessment shall be done in accordance with standards of care and standards of nursing practice.

4.4. Data collection will involve the patient, family and other health care providers.

4.5. Pertinent data about patient's health status shall be collected utilizing appropriate assessment techniques, tools and equipment.

4.6. All patient cared in the hospital shall have their needs identified through appropriate nursing assessment. The initial assessment should be initiated upon admission.

4.7. Assessment data will be analyzed and will form the basis for formulating a nursing care plan.

4.8. Initial nursing assessment need to be done in each admission by the registered nurse.

4.9. Nursing assessment has to be performed periodically and/or any changes in the condition of the patient.

4.10. **NURSING ASSESSMENT**

4.10.1. Nursing initial assessment in PICU has to be completed and documented maximum within 6 hours of admission.

4.10.2. An initial nursing assessment of sick child comprising of pulse, BP, temperature and respiration, GCS and pain is made on arrival and should be documented.

4.10.3. The nursing admission assessment will be completed comprehensively.

4.10.4. Areas of assessment that do not apply to the particular child being assessed must be annotated "not applicable"

4.10.5. The assessment should comply the following:

4.10.5.1. Reason for admission/ chief complaint

4.10.5.2. Mode of arrival

4.10.5.3. Vital signs

4.10.5.4. Physical measurements (developmental age, length or height, weight and head circumference as indicated by the age and condition of the child)

4.10.5.5. Pain and its characteristics (use the pain scale to determine the pain)

4.10.5.6. Drug allergies details

4.10.5.7. Immunization status

4.10.5.8. Communicable disease details

4.10.5.9. Current medication details

4.10.5.10. Spiritual assessment/psychosocial status

4.10.5.11. Past medical and surgical history

4.10.5.12. History of blood transfusion

4.10.5.13. Physical assessment- systematic assessment which includes the following:

4.10.5.13.1. Head, eyes, ears

4.10.5.13.2. Skin, hair

4.10.5.13.3. Respiratory

4.10.5.13.4. Cardiovascular

4.10.5.13.5. Gastrointestinal

4.10.5.13.6. Obstetrics/ gynecology

4.10.5.13.7. Nutritional status

4.10.5.13.8. Psychological

4.10.5.13.9. Behavioral assessment

4.10.5.13.10. Anthropometric measurement

4.10.5.14. Active daily living details

4.10.5.15. Fall assessment tool (Humpty Dumpty)

4.10.5.16. Pressure ulcer risk assessment tool (Braden Scale)

4.10.5.17. Risk level and intervention based on pressure ulcer category

4.10.5.18. Patient/ family orientation

4.10.5.19. Discharge planning

4.10.5.20. Valuables if any

4.10.5.21. Post assessment referral tool

4.10.5.22. Suggested nursing diagnosis

4.10.6. During the initial assessment, any deviation within the normal values/ results has to be reported to the concern physician immediately.

4.10.7. Problems and needs identified within the nursing assessment will be clearly linked to an individual plan of care that identifies:

4.10.7.1. Actual / potential problems

4.10.7.2. Nursing intervention / rationale

4.10.7.3. Anticipated outcomes

4.10.7.4. Timeframe for evaluation of interventions

4.10.8. The nurse will include the patient and family in the care planning process whenever possible and this will be documented in the progress notes.

4.10.9. Where applicable, the nurse will generate referrals to other disciplines like nutritional needs, social and economic needs. Any other referrals that require physician orders will be then relayed and submitted.

4.10.10. Nutritional assessment should be performed to identify which patients are at risks, to be further referred immediately by the nurse to a clinical dietician.

4.10.11. Assessments of child must include identification of educational needs of both patient and family, and readiness and ability to learn, with clear documentation in the progress notes, nursing assessment, with a notation in the interdisciplinary patient/ family education Record as how these needs will be addressed.

4.11. NURSING REASSESSMENT

4.11.1. Reassessment should be done in pediatric critical care unit hourly.

4.11.2. Reassessment has to performed, if there is any sudden change in the patient condition and the findings has to be reported to the physician immediately.

4.11.3. Patients will be reassessed pre and post operatively, on transfer and when there is deterioration of improvement in condition.

4.11.4. The plan of care will be amended to reflect any changes in the condition of the patient following reassessment.

4.11.5. The patient's response to therapy as evidenced by the vital signs and laboratory values and changes in medication regimen.

4.11.6. Reassessment of the ventilated care plan will be done every 12 hours or more frequently if the patient's condition warrants.

4.11.7. The nursing staff reassess each patient to determine response to nursing treatment and interventions by the multidisciplinary team, as well as whenever there are significant findings.

## 5.0 PROCEDURE (S)

5.1. Identify the patient correctly to prevent misidentification.

5.2. Initial assessment has to be performed within the time frame of 6 hours.

5.3. Use appropriate assessment form, perform complete head to toe assessment to provide baseline data for further management.

5.4. Ensure the chief complaints are identified and interventions are done.

5.5. Reassessment has to be performed hourly and/ or continuous monitoring as per condition of the patient.

5.6. Any variations from the baseline data, complaints of patient have to be notified to the patient immediately.

5.7. All the desired findings have to be documented.

5.8. Appropriate intervention and the response of the patient to be documented in the nurse's notes.

## 6.0 EQUIPMENTS

6.1. Blank ink pen

6.2. Vital signs machine

6.3. Weighing scale

6.4. Height scale/ tape measure

6.5. Stethoscope

6.6. Examination gloves

## 7.0 ATTACHMENT/FORMS

7.1. Initial assessment form

7.2. Nurses notes

## 8.0 REFERENCES

8.1. King Fahad Hospital, Policy and Procedure for Admission and Discharge Criteria in PICU, Policy number IAR-PICU 38/3

## 9.0 REVISION CLAUSE

9.1. N/A